

COURSE NAME:
TRAUMA AND STRESS MANAGEMENT

Contents

INTRODUCTION	4
Trauma	5
TRAUMA AND THE BRAIN	5
Triune Brain Theory	5
Reptilian /Primitive Brain/Lizard Brain.....	6
The/Mid/ mammalian/Limbic Brain	6
The Neocortex/Frontal lobe/human Brain:	6
TRAUMATISED BRAIN	7
POST TRAUMATIC STRESS DISORDER(PTSD)	8
Intrusive thoughts/Reexperiencing	8
Avoidance symptoms.....	9
Arousal symptoms	9
Negative/irrational Thoughts	10
TRAUMA COUNSELLING APPROACHES	11
Psychological First Aid (PFA)	11
Brief Psychodynamic Psychotherapy.....	12
Cognitive Behavior Therapy	12
Self-Trauma Model (STM)	13
Principles.....	14
Safety and support.....	14
Therapeutic Feedback	14
Work Through.....	14
STAGE MODELS	15
Stage specific Model Herman (1997)	15
Stage oriented Model (Chu 1998).....	17
Stage 1: Self care	17
Stage 2. Acknowledgement.....	18
Stage 3. Functioning	18
4. Expression.....	18

Stage 5. Relationships	19
Mindful Self-Compassion	19
Pharmacotherapy.....	20
CONCLUSION.....	20
REFERENCES	21

INTRODUCTION

In the world over, traumatic events occur regularly. Examples are road accidents, floods, famine, deaths, serious medical conditions, politically instigated wars, abuse, family feuds, terrorism and rare cases of plane crashes. The latest event being the global COVID 19 pandemics. All these leave a trail of haplessness and despair in their wake. Therefore, a closer look at interventions is necessary to revamp the society.

Trauma can manifest itself directly through a myriad of symptoms like stress, depression, anxiety, anger outburst among others. However, it can also be complicated and camouflage itself in other ways like personality changes, difficulty in forming relationships, studies deterioration, diminished interests in things hitherto enjoyed, and psychosomatic symptoms like headaches, changes in blood pressure, diabetes among others. The causal events could have happened a week before, months or even years before during childhood. If the person does not deal with trauma, post-traumatic Stress Disorder develops. If faulty interventions the individual can stagnate and assume a pathological role.

However, with proper interventions individuals can recover fully and resume productive personal lives. In this course, I will begin by defining trauma and assessment of brain function in reaction to traumatic events. Later I will discuss the trauma treatment approaches. Most of the trauma interventions point to generally three stages of healing that the survivor passes through. They are: Safety and Stabilization, processing traumatic memories and Enhancing daily living (Integration with the community). Through a variety of techniques such as stress reduction, narration of events, relaxation, high challenge and High support in a therapeutic environment the client is able to deal

successfully with the trauma. However, it should be noted that trauma healing differs among individuals. There are those who heal faster depending on personal and environmental factors.

Trauma

Trauma is a painful, shocking, distressing emotional experience often has a lasting psychological effect. It threatens the basic belief of safety, predictability and trust. When one is suffering trauma, they fear that they are always in danger of such events (Laura,2020). They are no longer able to trust people or places similar to where the traumatic episode occurred. Trauma can be experience firsthand, watched or having a loved one affected.

TRAUMA AND THE BRAIN

Triune Brain Theory¹

The theory was developed by Dr, Paul Maclean, an American neurologist in the 1950s. According to his study though we have one mind it is divided into 3 brains, incidentally they operate simultaneously and may not necessarily agree. Each brain has its goal. At times totally different things. The three brains are **Lizard brain** whose main concern is survival, kill-or- be- killed mentality, flight, fight or freeze response. **The mammalian brain** that is responsible for emotions. To reproduce, eat and dominate are the primary function. The third is the **frontal lobe** which is the seat for judgement. This is concerned with rational objective thinking, morality and spirituality. It is concerned with truth, justice, learning genuine love and creation like creative arts).

¹ <http://www.reconnect-to-your-core.com/The-Triune-Brain-Theory.php>

To explain how brain is related to trauma, I will use the Triune theory of Brain by Paul MacLean. In 1950s the US Neurologist discovered that our brain is divided into three parts according to evolution. The Three parts are: autonomic

Reptilian /Primitive Brain/Lizard Brain

The sole motivation for this part of the brain is survival. It is responsible for the reflex reactions. For example, when it is cold, what happens to the skin? It develops goose bumps and when it is hot, the veins dilate to lose heat. When one has not eaten, the stomach may rumble, yawns, and feels the pangs of hunger. These responses are the as a result of the reptilian brain. Why it is called the reptilian brain, it's because we humans share this brain with the lizards and the other reptiles. It is purely based on instincts. (Bath 2005)

The/Mid/ mammalian/Limbic Brain

The main motivation for Limbic brain is to reproduce and get what it wants. It is also the seat for emotions like joy, sadness, happiness, anger, jealous. distraught etc. It I also responsible for memories of events, habits and attachments. We humans share this part of the brain with mammals. That is why it is called Mammalian brain.

The Neocortex/Frontal lobe/human_Brain:

This is the front part of the brain which is responsible for Language, abstract thought, imagination, consciousness, reasoning and rationalizing. Neocortex regulates the primitive parts of the brain. For example, the reptilian brain will inform the person that he is hungry and he needs to eat right away but the Frontal lobe will assess the situation and tell the person that it is not time to eat or the food place is far therefore he can wait. Same case applies to the mid brain in need of sex immediately. The frontal lobe will inform whether it is appropriate time or not, if not appropriate

time then the mammalian brain will have to wait to fulfill that satisfaction. In an ideal situation we are able to think clearly, make good decisions, and be aware of ourselves. But as we progress in the course, we will see what happens when we are overwhelmed and the frontal lobe is affected.

Bath, (2005)

TRAUMATISED BRAIN

The brain is the organ that is responsible for marking traumatic situations in our lives. What happens when an incidence is labeled in the brain as traumatic? What happens to the three brains that otherwise coexist peacefully, when trauma strikes?²

Between the lizard brain and the Limbic brain, there is a part called fear Centre, Amygdala. It is outside of conscious awareness control. Its primary job is to receive all information. Through the senses: Touch, smell taste, and auditory. If it senses the information as dangerous and threatening it produces fear in us and thus, we remain active or vigilant. If the threat is gone then we revert back to normal. However, if the issue is too threatening the thinking center (frontal lobe and the emotional regulation center is deactivated. The fear center is highly activated. The body is set in motion to either of the three responses: Fight, flight, or freeze. As a result, you may experience chronic stress, vigilance, reexperiencing the trauma, have a hard time falling asleep, difficulties in concentration and attention, failure to think clearly and becomes incapable of managing emotions. This is because the auto pilot is in control, the frontal lobe and the mammalian brains are deactivated. If the traumatic incidence is dealt with the system reverts back to normal. Those feelings and emotions are normal after experiencing a traumatic incidence. However according to

² <https://www.youtube.com/watch?v=4-tcKYx24aA&t=56s>

DSM 5, when the symptoms persist at least a month then Post Traumatic stress Disorder (PTSD) is diagnosis is made. As Shallcross (2010) points out. If not, the person may develop.

POST TRAUMATIC STRESS DISORDER(PTSD)

According to Diagnostic Statistical Manual 5 (DSM) of mental Disorders, PTSD is characterized by 4 Broad categories of symptoms. These may occur up to several years after the traumatic incident. They are:

Intrusive thoughts/Reexperiencing

The survivor may experience images relating to the trauma, nightmares, daydreaming, flash backs, sense that the whole episode is reoccurring now, distress at events, places or symbol reminding of the trauma. The irony is, the physical trauma(injuries) may have long been healed.

A case study

Miss X had an automobile accident 5 years ago. All seemed normal after a 4 months hospitalization. However lately she has been having dreams being involved in an accident. She also occasionally during the day have persistent thoughts about her death in an accident. This has tampered with her normal functioning at work place. She is unable to concentrate and recently she received a warning letter from her boss owing to incomplete or late assignments. She feels she does not have any control about it and she is not able to communicate her problem with the boss. An earlier attempt to talk about it to a colleague, she was told, "You need to forget about the accident and move on, after all you recovered from the injuries."

Avoidance symptoms

The survivor also experiences the need to forget or escape the reality. For example, they may avoid people or places and thoughts that remind them of the incident. They may have loss of interest in discussing the incident, or the area that triggered. For example, in the above example of the accident, the survivor may not want to talk about where she was going or events prior to the incident. In real sense they may experience difficulties in recalling the details of the traumatic incident. In extreme cases they may temporarily pass out when the thoughts for example, a counsellor friend of mine during supervision of her sessions, she mentioned a case of her client, girl who had been gang raped. One of the challenges the counsellor reported to me was a situation where the client passed out for like 5 minutes during the session. Upon enquiring from the client, she said that she experiences fainting every time she tries to talk about the incident. That I responded to the counsellor is a classic example of extreme avoidance. The reality is too overwhelming to the frontal lobe so the lizard brain takes over and numbs the whole brain to avoid the painful reality even though for few minutes so as to cope with the reality. The irony is at the same time there is reexperiencing yet the person wants to avoid the thoughts. This can really cause emotional turmoil to the survivor.

Arousal symptoms

The survivor also experiences Arousal symptoms characterized by hypervigilance characterized by expectation of danger anytime and sleep disturbance. The client becomes oversensitive and suspicious of strangers. If maybe the survivor had experienced a burglary, they may wake up several times at night to confirm the door is properly locked. On the above cases of accident and rape, the survivor may swear to never board that kind of vehicle again and may never walk on that

path again respectively. The survivor experiences exaggerated startle response, irritability and angry outbursts to others who are not related in anyway with the incident. For example, soldiers from war zone are reported to have “taken cover” on a mere loud bang of the door by the wind. On realizing it was not a gunshot from the enemy, they may shout at the family member who left the door open at the masses of the wind. An unexpected immediate loud volume of the TV may scare them.

Negative/irrational Thoughts

Following a traumatic incident, the cognitions are affected and can exhibit either of the following thoughts. Persistent distorted cognitions of the causes of the incident. For example, a curse or bad omen pronounced by somebody, they may also blame others or self irrationally. For instance, they may blame the corrupt police who allow unroadworthy vehicles on the road. While it is important to have road worthy vehicles on the road probably the accident did not have anything to do with road worthiness of the vehicle, Maybe the vehicle had been slammed into by a speeding motorist. The survivor may experience persistent negative emotional state like “I will never travel to that place anymore,” “I will never get married.” “Sex is evil,” “all men/women are beasts” among others. At times the victims blame themselves. Like Patrack & Hedge (2002) point out that the victims of rape tend to even blame themselves and therefore may not even report the incidence. I agree with that regarding the above supervision case of the rape case, the survivor never mentioned that to anyone. First time in five years did she mention to the counsellor only at prompting of the latter. So, if my friend never bothered to talk to her, the girl would continue hurting from the inside without anybody never realizing. Such are the cases that lead to suicide or other complicated psychological dysfunctions like acute depression. Likewise, in a case of death of an infant the mother may live to blame themselves. A couple clients of mine came for therapy on account of

their two months old infant died. The mother blamed herself that she went to wash clothes instead of staying with the child, who died in his sleep. Other members of the family had all sorts of irrational reasons as to why the child died. Since they came for therapy three months after the burial, the mother told me that she felt like she was leaving the baby a “lone”. On enquiring what she meant, she told me that since the baby was buried in their compound, so anytime she left home, she felt guilty that she is leaving the baby alone. She also felt the baby is being “rained on” outside.

It is important to note that several psychological disturbances can take shape of any of the above PTSD symptoms. Therefore, it is paramount to note that according to DSM 5, all the symptoms must be there for the diagnosis to qualify PTSD.

TRAUMA COUNSELLING APPROACHES

Psychological First Aid (PFA)

It is advisable if possible, within 72 hours of an encounter with a traumatic event for survivors to undergo PFA. This is a recommendation by World Health Organization (WHO, 2011) and endorsed by various psychological Associations in the world led by American Psychological Association (APA). PFA is primarily designed for interventions in community trauma like floods, war, terrorism, fires, pandemics like COVID 19 among other communal adversities. However it can still be customized to deal with individual cases. For intervention for a traumatized clients advocates for four principles (**P3L**) **Prepare:** The provider needs to learn the crisis event, available resources and also learn about the safety and security concerns. **Look:** The responder need to observe for safety both his and the survivors, observe for the people with obvious urgent basic needs, observe for people with serious distress reactions and assist them first. **Listen:** The responder may need support, ask about their concerns and listen to people telling the story of the incident to help them feel calm. And lastly **Link:** The service provider needs to help people address

basic needs and access services like medical cases. Help people cope with problems, provide information and connect people to their loved ones and other necessary referrals like medical needs and psychosocial needs. The PFA providers are not necessarily counsellors but they could be community health workers trained in provision of psychological First Aid. Therefore, one of the referral destinations is to see a counselor.

Brief Psychodynamic Psychotherapy

This focuses on early life experiences of traumatic incidents. With the help of a calm, empathic and nonjudgmental therapist, the client goes back in history to retell the traumatic experiences (Brailsford (2007)). For example I had a client who had been being defiled by an uncle, she was then in preschool. This caused a lot of anxieties later in life. In fact, clients with past traumatic incidences present other issues in therapy like difficult forming relationships, marital problems among others. Therefore, talking about the traumatic incidence helps the client to gain control of things, gain more effective thinking strategies and gain ability to manage intense emotions successfully. Like my client later reported to me that she had been able to form meaningful relationships with men. During the session she was able to gain insight that she was relating with all men from the perspective of the small girl with the abusive uncle. She was able to gain mastery over the situation and overcome the internalized helplessness where she was fixated at age 4. Psychoanalysis She is able to take control of her life consciously.

Cognitive Behavior Therapy

Cognitive Behavior Therapy targets the maladjusted thoughts, emotions and behavior's the main aim of CBT is to weaken the connections between maladjusted emotions, troublesome thoughts

and their guided behavioral reactions (Brailsford 2007, Scott & Stradling (2006)). The counsellor uses an array of techniques such as exposure therapy, systematic desensitization, Anxiety management, and stress inoculation therapy. Traumatic experiences usually hamper the emotions and thoughts. Remember the overwhelmed deactivated frontal lobe and the emotional regulation by the reptilian brain. The four signs of PTSD are targeted by CBT. First, the trauma replays (Reexperiencing) cause distress to the client. Second, so he tries to inhibit them by avoidance but they are too strong, they keep popping up in the way of, three, hyperarousal. This causes extreme distress since the client is unable to “forget” to the client and results to, four negative cognitive distortions in order to cope. For example, presence of any man triggers fears of rape to a rape survivor. According to Lees-O.R. & Kelly, K. (2017) This means that the client matches new information with the old inner models. That is any male is perceived as a rapist. So, the key intervention by the therapist is to challenge the client match new information (any man) with new model (Not a rapist). The therapist helps the client to talk about the story (To deal with avoidance and arousal) in order to reconcile with what happened. The client is helped to deal with cognitive schemas previously held and subsequently learning new behaviors or relearning old adjusted behaviors. For example, the client who had the grisly road accident never feared cars before but after the accident, she developed the schema that it is unsafe to ride a car. So the counsellor through cognitive behavior therapy techniques will help the client develop a schema like, Though I had an accident it does not mean every time I ride in a car I will get an accident.

Self-Trauma Model (STM)

This was developed by Briere (1996). This is a type of treatment that calls for active participation of the client in order for healing to be realized. The therapeutic environment is very key in

confrontation of pain and fear so as the client can be fully emancipated. It is a blend of humanistic, psychodynamic and Cognitive behavior theories. There are three important principles in this treatment: *Respect, Positive Regard and Assumption of growth.*

Principles

Safety and support

Note that when clients come in session after a traumatic incident, they are harboring all sorts of negative emotions: Stress, anxiety, helplessness, desperation, depression, self-guilt, blame anger, sadness, confusion among others. Therefore, feelings of safety and support is key in helping them develop a sense of self which helps them to self-regulate and be able to rely on their internal resources during the time of stress. This also builds a client positive sense of self and allows them to move from a state of hypervigilance to calmness. A consistent supportive therapeutic environment helps the client explore their inner self safely

Therapeutic Feedback

Therapeutic feedback is means, the responses and the observations of the therapist to the client. At This constitutes listening attentively, encouragement, motivation, empathic questioning supportive challenge and psychoeducation. In such an environment, the client is able to understand the situations they are in. States of self-defeating and destructive thoughts and behaviors are discarded and replaced with adjusted one's clients gain control of themselves because they are no longer acting from the unconscious irrational autonomic brain. As therapy progresses, Clients develop increased self-nurturance, confidence and self-support.

Work Through

Having made sense of what is going on in his life, the client is helped to work through the positive areas previously avoided or thought risky for example in the above case of my client who had

always feared opposite sex relationship, can explore how to go about it. They may be able to confront an abusive parent or boss. This is made able by the environment of unconditional Positive Regard provided by the therapist. At this point the therapist can introduce roleplay or empty chair technique where the client explores difficult areas in a safe environment. The therapist explores traumatic materials within the demarcated **therapeutic window**. This is the area between being highly affected by the disclosure and excessive avoidance that will cause stagnation. So the therapist works within tolerable intensity by the client. This means the therapist's interventions should not cause too much emotional overwhelm neither should they be too low challenging hence causing client stagnation Briere (2002). This can be likened with the theory of scaffolding coined first by Vygotsky (1978) as described by Devereux, L.& Wilson, K. (2013). The theory postulates that for learning to be effective, there should be high challenge and high support to the learner by the educator. Self-trauma concept of **therapeutic window** seems to agree that the client needs high challenge and high support for them to worth through the emotional impasse they have been thrown into by the traumatic event.

STAGE MODELS

Stage specific Model Herman (1997)

Herman 1997, explains that trauma recovery goes through three main stages:

Safety and Stabilization:

The counsellor focuses on the calmness of the client. The client is helped to shift from the unpredictable danger mode to safety, dissociated trauma to acknowledged memory and stigmatized isolation to restored social connection. The client is helped to restore trust in others by validating her experience. The client is allowed to have control in the therapeutic setting where the counsellor acts as facilitator. The latter however guards against being a rescuer and patronizing

should adopt a neutral therapeutic stance to help the client see themselves from all fronts. The process helps the client to reestablish themselves and gain control over anxiety, hyperarousal and vigilance. Collaborative therapy is enhanced at this stage where the client leads the way as the therapist act as the coach.

Stage 2: Remembrance and mourning

The main aim of this stage is for the client to tell the whole story of the traumatic experience. Through the behavioral techniques of flooding and testimony, the client is helped to transform the snapshots they have regarding the trauma to and the real picture. However, the balance between too overwhelming disclosure (disinhibition) and avoidance is negotiated because too much of the former can overwhelm the client and too much of the latter can make the client to stagnate. The client is allowed to grieve the loss. Therefore, the counsellor should be very well versed with grief counselling models for example Kubler Ross stages of loss and grief. In remembrance the therapist needs to take note of all the senses regarding the trauma (Briere 2002). For example, ask questions: What can you see, smell and touch, hear and taste? And The therapist needs to reassure the client about the normal reactions at this time and help them discard the pathological reaction. Some clients may see mourning as allowing the perpetrator “victory”, as a sign of defeat, sign of lack of faith for those who are strong in faith. Sometimes the significant others albeit well-meaning do not help much with platitudes. However, mourning should be seen as a sign of strength and path to healing. This however does not mean that justice should not be sought.

3.Reconnection

Having gone through successful safety and remembering and mourning the client can easily reconnect with the new self. They reconcile with the past before trauma. It is important to note that they can never forget what happened but they remember without intense pain and hurt at the same time they are able to move on with life. Survivors recovering from childhood sexual abuse, the

new identity may feel like a second adolescence. This is because they got stuck at the age where they were traumatized. Such that before therapy they have been relating to the world from the traumatized child point of view. Briere (2002) emphasizes that they transit from a victim to survivor. The hitherto sense of powerlessness is replaced with control and productivity. The survivor may rebuild relationships new or old. She or he may decide to confront the aggressor in bid to liberate elf from the trauma for example, a very moving story aired by Kenya diaspora TV (n, d) on you tube about a lady who testified being a very difficult childhood all the way up to adulthood. She says she got into marriage at 11 years and she lived in hell with her husband and deserted by her mother and siblings. However, after many years of counselling and getting saved, today she is a minister of the word of God. She is a pastor who is using her story to encourage other survivors and to discourage such ills being meted to anyone. Her story is a typical example of somebody who was a victim turning survivor and a conqueror. She is now the master of her own destiny. There is a gospel artist in Kenya who has sang about his beloved mother who died when he was nine years. In the song the artist rhymes very well the 9 months and nine years(in Kiswahili language) he spent with the mother in a very artistic way. This has helped him voice his traumatic experiences in a constructive way. The song has actually won awards. Others may reconnect through community service like helping destitute children.

Stage oriented Model (Chu 1998)

The stage-oriented model by Chu, has got five stages with acronym SAFER (Self-care, Acknowledgement, Functioning, Expression, Relationships)

Stage 1: Self care

This stage focuses on eradicating the survivor's feelings of unworthiness and has a sense of identity and control. A sense of hope and calmness is instilled. The grounding techniques such as deep

breathing, sensations, squeezing a ball or rubbing a stone to help the client overcome intrusive thoughts and reorient living in the current moment. Others are writing a life story, journal and restructuring automatic thoughts(<https://www.therapistaid.com/>)

Stage 2. Acknowledgement

The client is helped to acknowledge that the trauma happened. This confirmation is key to recovery. Otherwise continuous denial for keeps the client stagnated. Therefore, the counsellor should never be an ally of the client in denial, avoidance, or flawed states of personal defectiveness the client should be reassured that they were not responsible for the incident.

Stage 3. Functioning

At this stage clients are encouraged through scaffolding technique of high challenge, high support to restart their functioning. For example, the client can resume work or school if maybe they had stopped. They are encouraged to quit dwelling on the past at this stage they could still be anxious about their new selves, for example if they fell pregnant or contracted infections through say rape, or if they lost a limb through horrific accident or they lost a loved one. The counsellor can use Greenberg's (2020) several anxiety reduction techniques such as maintaining a routine, dwelling on the positive aspect of themselves constantly paying attention to situations that make them feel safer, think about the people in their lives who support them,

4. Expression

To free themselves hitherto unspeakable aspects of trauma, clients are facilitated ways of expressions. For example, poetry, drawing, dancing can help the client express traumatic aspects in constructive ways

Stage 5. Relationships

The client practice reconnection with family and friends. The therapist needs to model a corrective emotional relationship for the client. The client can participate in interactions which were hitherto viewed with anxiety. For instance, remarriage in case of a divorce as a result of abuse. However, the client needs to continue with therapy as he encounters new challenges. However, the therapist needs to help client with skills to face new challenges as they are inevitable.

Mindful Self-Compassion

As stated elsewhere in this course, trauma forces the brain to go to either fight, flight or freeze mode. (This is normally accompanied by a host of other emotional reactions like depression, guilt, anxiety, lethargy among others) With these the client experiences Self-criticism, Isolation or rumination respectively. Mindful Self compassion exercises helps the client to replace them with Self kindness, common Humanity and mindfulness. (Neff, &Garmer (2013), <https://self-compassion.org/>) For example The client can shift from ,”I am responsible for what happened so I am stupid(Fight)to Whatever happened was not my fault so may I have peace(Self Kindness).”A shift from, “How could they do that to me, I will never get married again, All men are beasts(Flight),” to “Whatever happened to me just confirms that I am human like other human beings who suffer that. My reactions too are human. Any one in my place will just behave the same, however there are marriages that work, there are men who are good (Common Humanity)”and a shift from. “I cannot stop thinking about this, I am overwhelmed (Rumination)” to “The events of that fateful day cannot escape my mind but I will remember with care to myself (Mindfulness)”

Pharmacotherapy

In some instances when the trauma symptoms are excessive medication can also be administered. For example, emotional numbness, depression and anxiety Psychosomatic illness such as High blood pressure, stroke, Diabetes, general malaise should be treated by a medical doctor. Murray(2019), describes the list of pharmacological interventions³, antidepressants and mood stabilizers that need to be administered according to the severity of the emotional disturbance. However, it is important to note that pharmacology should always be used along psychotherapy. That is, pharmacology is never enough alone. It just addresses the symptoms.

CONCLUSION

Trauma can be very devastating to the survivors. It normally has devastating effects to the survivor for the rest of their lives. Unfortunately, while some symptoms are visible, one may go on with life oblivious of what they are suffering from since it may present symptoms seemingly unrelated to the event. Therefore, significant others need to be fully vigilant with changes of behaviors or out of ordinally behavioral presentations. Nevertheless, if detected one can be treated through various psychotherapeutic interventions, and pharmacological interventions (if necessary) and later can lead normal lives. Otherwise living with PTSD can be frustrating to both the sufferer and the significant others. Owing to the camouflaging and comorbidly associated with trauma, lot of sensitization needs to be done to enhance psychologically healthy society.

³ <https://www.uptodate.com/contents/pharmacotherapy-for-posttraumatic-stress-disorder-in-adults>

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